



Policy

Searching of Patients Belongings and Property

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Functional subgroup

Summary

The East Metropolitan Health Service (EMHS) has a responsibility to provide a safe environment for patients, visitors and staff. EMHS will take all reasonable steps to ensure that admitted patients, detained persons and persons presenting at a mental health service do not present a risk of harm to themselves or others by having inappropriate items in their possession and/or bringing them into mental health inpatient units.

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Status Active

1. Background

The East Metropolitan Health Service (EMHS) has a responsibility to provide a safe environment for patients, visitors and staff. EMS will take all reasonable steps to ensure that admitted patients, detained persons and persons presenting at a mental health service do not present a risk of harm to themselves or others by having inappropriate items in their possession and/or bringing them into mental health inpatient units.

EMHS will also strive to ensure visitors to mental health units are not able to bring inappropriate items into mental health inpatient units.

Inappropriate items include dangerous items, alcohol and drugs, items which are clinically indicated as potentially causing harm to financial status or reputation to a patient, and items that the admitting or treating psychiatrist considers to be otherwise inappropriate

1.1. Dangerous Items

Dangerous items are those items which:

Are hazardous to the health or safety of any person; or
Are capable of causing serious damage to any property.

These items may include the following (not intended to be an exhaustive list):

- Firearms
- Prohibited and controlled weapons
- Articles with the potential to be used as weapons, including sharp or blunt objects
- Potential ligatures (scarves, belts, shoelaces, headphone cords, telephone chargers)
- Plastic bags
- Highly flammable materials
- Ignition sources, e.g. cigarette lighters
- Hazardous chemicals and poisons

Refer to Legislation [Weapons Act 1999](#)

Refer to Legislation [Firearms Act 1973](#)

Refer to Legislation [Mental Health Act 2014](#) Section . 164(2)(b). (Seizure of articles) This section applies in relation to the seizure from a person of an article under section 162(2)(b).an article, including a drug that is prescribed for the person, that may pose a serious risk to the health or safety of the person or another person;

1.2. Alcohol and Drugs

Intoxicants are not permitted in mental health inpatient units. These include:

- Alcohol
- Illicit drugs
- Solvents and inhalants, e.g. petroleum, glue, acetone, amyl nitrite, spray deodorants (if clinically indicated)

- Prescription and over-the-counter medication (these must be declared and surrendered to clinicians upon entry into the unit)

Refer to EMHS [Alcohol and Prohibited Substances](#) Policy.

Refer to EMHS [Testing Mental Health Inpatients for Alcohol and or illicit Substance Use and / or Medications not Prescribed by the Treating Team](#). Policy

Refer to Legislation [Misuse of Drugs Act 1981](#).

1.3 Items Which Would Assist in Determining Status

A person conducting a search may seize “any item which that person believes is likely to materially assist a psychiatrist” in determining a question in relation to the searched person that is likely to arise for determination

That is, a person conducting the search may seize any item which would assist a psychiatrist in determining whether the searched person should be made an involuntary patient.

Refer to [Mental Health Act 2014](#) Section. 164(2)(c). (Search and seizure powers)

1.4 Items Causing Harm to Financial Status or Reputation

Principle 9 of the Charter of Mental Health Principles provides that mental health services must recognise the range of circumstances that influence mental health and wellbeing, including relationships and financial circumstances.

Refer to EMHS [Rights of Patients in Mental Health Apx A: Charter of Mental Health Care Principles](#) policy

Certain items potential present a risk of significant harm to a patient, including harm to the patient’s financial status or reputation. These items might include:

- Mobile telephones
- Cameras
- Laptop computers and tablets
- Other electronic devices

Further, a psychiatrist may order restrictions on an involuntary patient’s access to letters and electronic communications (including email) and telephone calls: Mental Health Act 2014 s 262. A patient subject to such an order would be required to surrender any mobile telephone devices upon re-entry to an inpatient unit (for example, upon returning from leave).

Refer to [Mental Health Act 2014](#) Section. 262. (Restrictions on freedom of communication)

A psychiatrist must review the order every 24 hours, at which time it may be renewed or revoked.

- The order will lapse if not reviewed within 24 hours.
- A record of the order is to be made in the patient health record.

- A copy of the order is to be given to the patient's personal support person and the Mental Health Advocacy Service must be notified.

Refer to EMHS [Notifications in Mental Health](#) Policy.

Refer to Section . [Mental Health Act 2014](#) 259(4)-(5). (Personal possessions)

2. Inappropriate items not permitted within Mental Health Inpatient Units

Sites should employ methods to notify potential patients and visitors that inappropriate items are not permitted within mental health inpatient units. These methods might include signage or information brochures.

Staff should notify patients prior to admission that inappropriate items are not permitted within mental health inpatient units. Patients and carers should be provided with written information explaining the items that are not permitted.

Information should be presented in a language the patient can understand.

Refer to [MP0051/17 Language Services Policy](#).

Refer to EMHS [Aboriginal Cultural Considerations Policy](#)

2.1 Items that are otherwise inappropriate

A treating psychiatrist or person in charge of an authorised hospital may restrict those articles of “personal use, wear or ornament” which in his/her opinion it would not be appropriate for a patient to use in an inpatient unit

(Appendix A)These guidelines are to be read in conjunction with EMHS: 153 Searching of Patients Belongings and Property

These guidelines set out the procedures for the search under the Mental Health Act 2014 of:

- Persons detained to enable an examination to be conducted by a psychiatrist
- Persons presenting at a mental health service for treatment
- Patients upon entry to, or while within, a mental health inpatient unit

Search of detained persons, persons presenting at a mental health service for treatment and mental health patients

3. Search

The prevailing consideration is the modesty and dignity of and respect for the person. Staff must be sensitive to issues of:

gender

- sexuality

- religion and culture
- past history of torture or trauma
- past history of sexual abuse or sexual violence
- past history of violence and aggression
- past history of self-harm

Prior to search

Identify self to person and explain the reasons for the search

Advise the person that the search can be undertaken in the presence of the patient's nominee if they wish:

- Explain the search process to the person
- Give the person the opportunity to declare whether they have any inappropriate items on their person or stored with their belongings and surrender such items to SMHS staff
- Take the person to a private area for the purpose of the search

Level 1: Self-search

In the first instance person should be given the opportunity to perform a self-search. A self-search entails

- Remove outer layer of clothing
- Remove hair ties and run fingers through hair
- Remove and turn out shoes
- Turn out pockets
- Show nothing is concealed in the mouth or under the tongue

Following the self-search, staff should

- Instruct the person to remove inappropriate items and place upon a table
- Store or dispose of items appropriately
- Offer person a debriefing
- Document the search using Form 8A and give the record to the person in charge of the mental health service

Level 2: Pat-down search

Where staff continue to hold concerns that the person has inappropriate items in their possession, a pat-down search should be performed.

- by clinical staff of the same gender as the person, if practicable
- Consider the need to wear protective clothing or equipment
- Explain the search process
- Seek the person's consent prior to searching
- Take the person to a private area for the purpose of the search

A pat-down search entails:

- In the presence of two staff, conduct a pat down of pockets and any areas that could be used for concealing items
- Avoid genitals and breasts unless previously agreed with senior registered nurse and conducted in the presence of clinical staff of the same gender as the person, if practicable

Following the pat-down search, staff should

- Instruct the patient to remove inappropriate items and place upon a table

- Store or dispose of items appropriately – complete Form 8B
- Offer person a debriefing
- Document the search using Form 8A and give the record to the person in charge of the mental health service

Level 3: Partial removal of clothing search

Where staff continue to hold concerns that the person has inappropriate items in their possession, a partial removal of clothing search should be performed

- by clinical staff of the same gender as the person, if practicable
- Consider the need to wear protective clothing or equipment
- Explain the search process
- Seek the person's consent prior to searching
- Take the person to a private area for the purpose of the search

A partial removal of clothing search entails

- In the presence of two staff, ask the person to remove all clothing except underwear so that the clothing can be examined by staff to ensure it does not contain inappropriate items.
- Ensure that only one item of clothing is removed at any one time (i.e. the person is not to be completely unclothed at any time).

Following the partial removal of clothing search, staff should:

- Instruct the person to remove inappropriate items and place upon a table
- Allow the person to dress or provide assistance if required
- Store or dispose of items appropriately – complete Form 8B
- Offer person a debriefing
- Document the search using Form 8A and give the record to the person in charge of the mental health service

Search of visitors to mental health inpatient units

Visitors to a mental health inpatient unit may be asked to perform a self-search where staff hold a reasonable concern that they have inappropriate items in their possession.

Prior to self-search

- Explain the self-search process to the visitor
- Ask the visitor to consent to the self-search
- Advise the visitor that the self-search can be undertaken in the presence of the visitor's nominee if they wish
- Give the visitor the opportunity to declare whether they have any dangerous items on their person or stored with their belongings and surrender such items to SMHS staff
- Take the visitor to a private area for the purpose of the self-search

Self-search

A self-search entails

- Remove outer layer of clothing
- Remove hair ties and run fingers through hair
- Remove and turn out shoes
- Turn out pockets
- Show nothing is concealed in the mouth or under the tongue

Following the self-search, staff should:

- Instruct the visitor to remove inappropriate items and place upon a table
- Store or dispose of items appropriately – note that self-search of visitors is not governed by the Mental Health Act 2014 and there is no requirement to complete a Form 8B

Visitors should not be subjected to a pat-down search or a partial removal of clothing search under any circumstances. Where a visitor self-search has been conducted but staff hold a reasonable concern that the visitor is in possession of inappropriate items, the visitor should be refused entry to the inpatient unit. The patient's treating psychiatrist should be notified and consideration given to liaising with security to restrict the person from visiting the patient.

Search of person's belongings (patient, detained person, person presenting at mental health for treatment or visitor)

Where staff hold concerns that a person has inappropriate items stored with their belongings, a search of belongings should be performed

For reasons of modesty and sensitivity, it is preferable that a search of a person's belongings be performed by clinical staff of the same gender as the person, if practicable

- Consider the need to wear protective clothing or equipment
- Explain the search process
- Seek the person's consent prior to searching
- Take the person to a private area for the purpose of the search
- Remember that this type of search must not involve any bodily contact with the person

A search of belongings entails:

- In the presence of two staff, ask the person to empty containers (such as pockets, bags or backpacks) and ask him or her to disclose any inappropriate items
- Carefully open containers to inspect for inappropriate items
- Never put hands in blindly to areas that you cannot see or see into

Following the search of belongings, staff should:

- Remove dangerous items
- Store or dispose of items appropriately – complete Form 8B (note that self-search of visitors is not governed by the Mental Health Act 2014 and there is no requirement to complete a Form 8B)

Visitors should not be subjected to a search of their belongings without their consent under any circumstances. Where a visitor does not consent to a search of their belongings but staff hold a reasonable concern that the visitor is in possession of inappropriate items, the visitor should be refused entry to the inpatient unit.

3.1 Search of the patient prior to admission or returning to the unit following leave

Where staff hold a reasonable concern that a patient is in possession of inappropriate items, a search of the patient and/or their belongings might be necessary.

There should be no routine search of a particular patient. Staff must hold a reasonable concern that the patient is in possession of inappropriate items.

A search must be carried out by an authorised person and witnessed under the Mental Health Regulations 2015. An authorised person is:

If the search is to be carried out at a hospital: the person in charge of a ward or a person immediately authorised by the person in charge of a ward;

If the search is to be carried out at a mental health service other than a hospital: a staff member of the mental health service;

If the search is to be carried out at a place other than a mental health service: a health professional at the place.

Refer to Legislation [Mental Health Regulations 2015](#) (Regulation 9) Persons authorised to exercise search and seizure powers(Act s. 161) A person authorised to carry out a search may use reasonable force in doing so.

Refer to [Mental Health Act 2014](#) Section.163(5)(c). (Conduct of search)

Security staff are lawfully authorised to give reasonable assistance to mental health service staff carrying out a search, and may use reasonable force in doing so.

The staff member should identify themselves to the person, explain the reasons for the search and ask for the patient to consent to the search.

The person conducting the search may photograph part of or all the search while it is being conducted

Refer to [Mental Health Act 2014](#) Section 172 (Reasonable assistance and reasonable force authorised)

Refer to EMHS [Clinical Photography and Videography](#)

Photographs should be stored in the patient's medical record in accordance with the EMHS Clinical Photography and Videography Policy.

4. Dealing with inappropriate items during search

Firearms, prohibited weapons, controlled weapons and illicit substances Staff should provide these items to site security services for surrender to police. The process must be fully documented according to site-based protocols, including:

Documenting the confiscation in the patient's health record; and

Obtaining a signed and dated receipt from police when the items are transferred into their possession (this is done via site security services).

Liaise with site security services to notify police that the items have been removed during a search. The identity of the person from whom the items were removed should not be disclosed to police except with the prior authority of a member of the site's medical or nursing executive.

Refer to [Mental Health Act 2014](#) Section. 166. (Dealing with articles seized when person apprehended)

In the event that staff are threatened with a weapon, a Code Black Bravo should be activated.

Potential weapons, alcohol, tobacco and other items of personal property

These items should be logged by the health service and stored. The items remain the personal property of the patient and are to be held on trust by the health service and returned to the patient upon discharge if determined appropriate by the treating team.

Refer to [Mental Health Act 2014](#) Section 167 (return of articles given or seized by Mental Health Service)

If the treating team determines that the items are not appropriate to be returned to the patient, they should be returned to the patient's personal support person.

Prescription and over-the-counter medication

Prescription medication for which the patient does not have a valid prescription should be handed over to Pharmacy for disposal.

Lawfully prescribed medication and over-the-counter medication should be confiscated and administered to the patient by the health service, or otherwise logged and stored then returned to the patient upon discharge if determined appropriate by the treating psychiatrist.

Refer to [Mental Health Act 2014](#) Section 164. (Seizure of articles)

5. If the Patient does not consent to a search of their person or belongings

Staff must request that patients consent to a search prior to carrying out the search.

If the patient does not consent to the search, the following processes should be followed.

Voluntary patients or persons presenting at a mental health service

Where a voluntary patient or a person presenting at a mental health service does not consent to a physical search, staff need to consider whether the patient can be admitted to the mental health service. Staff should calmly and rationally explain the reasons for the search and give the patient a further opportunity to consent. The person's family and carer may be involved in the discussion regarding consent, if appropriate. Staff might adopt other measures, such as consulting with the voluntary patient's treating psychiatrist and/or carer.

If all other measures have been employed and the person does not consent to a search, consideration should be given to refusing the person entry to the mental health service. This should be considered within the context of a clinical risk assessment.

Any decision to refuse admission to the mental health service and associated rationale should be documented in the patient's health record. Staff are to organise to follow up with the patient in the community.

Refer to EMHS [Mental Health Search Procedure](#)

Staff should consider whether the patient's decision to refuse to consent to a search, and therefore be refused admission to the mental health service, warrant a significant risk such that the patient's status should be reviewed (i.e. consider whether the person should be referred for examination to determine whether they should be subject of an involuntary treatment order). If all other measures have been employed and the voluntary patient does not consent to a search, the patient should be informed that the health service may lawfully conduct a search without the patient's consent and that it intends to do so.

The voluntary patient may then be searched in accordance with the Mental Health Inpatient Unit Search Procedure set out in Appendix A.

Refer to [Mental Health Act 2014](#) Section 163.(Conduct of search) Staff of a mental health service may request another person (e.g. security staff) to assist in carrying out the search. Both the mental health service staff and the security staff assisting may lawfully use reasonable force in conducting the search:

Refer to. [Mental Health Act 2014](#) Section 172 (Reasonable assistance and reasonable force authorised).

Involuntary patients or detained persons

Where an involuntary patient or detained person does not consent to a physical search, staff should calmly and rationally explain the reasons for the search and give the patient a further opportunity to consent. Staff might adopt other measures, such as consulting with the patient's treating psychiatrist.

If all other measures have been employed and the patient does not consent to a search, the patient should be informed that the health service may lawfully conduct a search without the patient's consent and that it intends to do so.

The involuntary patient may then be searched in accordance with the Mental Health Inpatient Unit Search Procedure set out in Appendix A.

6. Search of Minors

Inpatients who are minors (under the age of 18 years) should not be searched except in the presence of a chaperone.

Refer to EMHS [Chaperone](#) Policy

7. Search of Visitors

Visitors to mental health inpatient facilities must not bring inappropriate items into those facilities with them.

Visitors should be made aware by way of signage and verbal advice from staff that certain inappropriate items are not to be brought into mental health inpatient facilities.

Note: Level 1 Self-search is the only appropriate search to which visitors should be subjected. **Under no circumstances are visitors to be subjected to a Level 2 Pat-down search or a Level 3 Partial removal of clothing search.**

If staff have asked the visitor to conduct a Level 1 Self-search and remain reasonably concerned that a visitor is in possession of inappropriate items, the visitor should be refused entry to the inpatient unit and site security services notified.

If the visitor does not consent to a search of their person or belongings
Visitors who do not consent to performing a Level 1 Self-search or to a search of their belongings should be refused entry to the inpatient unit.

8 .Documentation

All patient searches must be fully documented in the patient's medical record.
Where a patient search has been carried out without consent, details of the search must be fully documented, including the reasons for the search and the patient's refusal to consent.

Complete MHA Form 8A: Record of search and seizure.

All items stored by the health service must be documented on the appropriate health service form.

All items removed by the health service for surrender to police must be entered into the appropriate property form / log / book and signed by the patient and a staff member, or (if the patient is unwilling or unable to sign) by two staff members. A copy should be placed in the patient's health record.

Visitor searches should be documented by way of a log of items removed from the visitor prior to entry into the inpatient unit.

Visitor searches where no items are removed should not be documented.

Complete MHA Form 8B: Record of dealing with seized article and enter into PSOLIS if relevant to clinical risks.

9 Compliance monitoring and legislative penalties

9.1 Compliance monitoring

Each EMHS site or service executive director is to ensure compliance with this policy.

All staff who perform searches of persons will ensure they are compliant with this policy.
Breach of this policy may result in disciplinary action under the Department of Health Misconduct and Discipline Policy and Guidelines.

Directors of Clinical Services, Directors of Nursing and Midwifery, Directors of Mental Health Services and Directors of Allied Health or equivalent are to ensure that this policy is communicated to all relevant staff.

Refer to MP0040/16 [Discipline Policy](#)

7.1. Legislative obligations

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

Staff must ensure that physical searches conducted under this policy are lawful.

An unlawful physical search of a patient constitutes an assault which pursuant to Criminal Code section 313 is punishable by imprisonment for 18 months and a fine of \$18,000.

Refer to [Mental Health Act 2014](#) Section. 163.

Refer to [Mental Health Act 2014](#) Section 172.

Refer to [Criminal Code Act Compilation Act 1913](#) Section. 313. (Common assault)

5.Legislation

Criminal Code Act Compilation Act 1913

Firearms Act 1973

Firearms Regulations 1974

Mental Health Act 1996

Mental Health Act 2014

Mental Health Regulations 2015

Misuse of Drugs Act 1981

Misuse of Drugs Regulations 1982

Weapons Act 1999

Weapons Regulations 1999

6.Additional references

7.2. WA Health policies

MP0040/16 [Discipline Policy](#)

Language Services Policy

7.3. EMHS policies

EMHS Alcohol and Illicit Substances in Mental Health Inpatients Unit Policy

EMHS Chaperone Policy

EMHS Clinical Photography and Videography Policy

EMHS Notifications in Mental Health Policy

EMHS Rights of Patients in Mental Health Appendix A: Charter of Mental Health Care Principles

EMHS Testing Mental Health Inpatients for Alcohol or Illicit Drug Use Policy

EMHS Appendix A Searches in Mental Health Appendix A: Mental Health Search Procedure

EMHS Aboriginal Cultural Considerations Policy

6.3 Other

Nil